INTEGRATED CLINICAL HYPNOTHERAPY FOUNDATION

***Formerly- California Hypnosis Institute Of India***

# INTEGRATED HYPNOTIC MODALITIES FOR BEHAVIORAL RESOLUTIONS (THEORY)

## LEVEL II

Office: (M) +91 983 340 2655/53

www.ekaa.co.in

admin@ekaa.co.in

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## OBJECTIVES FOR THE COURSE

At the end of this course the participants shall:

- Understand the various hypnotic modalities
- Understand the basics of Ericksonian hypnosis
- Understand body syndromes
- Understand basic emotion management techniques
- Learn the various hypno-diagnostic tools and their uses
- Learn the basics of dream therapy
- Learn the use of hypno-drama
- Learn the basics of age regression and age regression therapy
- Learn the theory for fear and phobia management
- Understand pain management
- Learn to use guided imagery for pain management
- Get an introduction to neuro-linguistic programming (NLP)

## 5 DAY –PROGRAMME SHEET

| Day 1 | | | |
|---|---|---|---|
| **From** | **to** | **Duration in mins.** | **Contents** |
| 0900 | 0930 | 30 | Registration & Introductions |
| 0930 | 1100 | 90 | Hypnotic Modalities |
| 1100 | 1130 | 30 | Tea Break |
| 1130 | 1200 | 30 | Ericksonian Hypnosis – Background & Indirect Approach |
| 1200 | 1230 | 30 | Indirect forms of Suggestions, Inverse Meta-Model Patterns and Additional Milton-Model Patterns |
| 1230 | 1315 | 45 | Body Syndromes |
| 1315 | 1415 | 60 | Lunch |
| 1415 | 1445 | 30 | Emotion Management Technique - Explanation |
| 1445 | 1530 | 45 | Demonstration & Self Practice of Emotional Management Technique |
| 1530 | 1630 | 60 | Dream Therapy |
| 1630 | 1645 | 30 | Tea Break |
| 1645 | 1730 | 45 | Dream Interpretation |
| 1730 | 1800 | 30 | Wrap up & Closure |

| Day 2 | | | |
|---|---|---|---|
| **From** | **To** | **Duration in mins.** | **Contents** |
| 0900 | 1015 | 75 | Hypno - Diagnostic Tools – Stages of Development |
| 1015 | 1045 | 30 | Corrective Therapy |
| 1045 | 1115 | 30 | Tea Break |
| 1115 | 1145 | 30 | Hypno-Aids and Ideomotor Response |
| 1145 | 1215 | 30 | Paris Window |
| 1215 | 1300 | 45 | Five stages of Loss |
| 1300 | 1400 | 30 | Lunch |
| 1400 | 1445 | 45 | Understanding Depression |
| 1445 | 1530 | 45 | Psycho-Drama / Parataxic Distortion |
| 1530 | 1615 | 45 | Empty Chair Technique / Hypno-Drama / Neuro-Pathways |
| 1615 | 1630 | 15 | Hypno-Drama in Sports |
| 1630 | 1700 | 30 | Tea Break |
| 1700 | 1800 | 45 | Age Regressions and Defence Mechanisms |

| Day 3 | | | |
|---|---|---|---|
| **From** | **To** | **Duration in mins.** | **Contents** |
| 0900 | 1000 | 60 | Fears vs. Phobias – Theory |
| 1000 | 1015 | 15 | Anxiety – Static & Free-floating |
| 1015 | 1100 | 45 | Overview & Reasons for Medical Hypnosis |
| 1100 | 1130 | 30 | Tea Break |
| 1130 | 1300 | 90 | Pain Management – Theory |
| 1300 | 1400 | 60 | Lunch |
| 1400 | 1500 | 60 | NLP Overview & Introduction |
| 1500 | 1600 | 45 | The NLP Process |
| 1600 | 1630 | 30 | Tea Break |
| 1630 | 1730 | 60 | Online Test |
| 1730 | 1800 | 30 | Certificate & Wrap Up |

# CHAPTER 1
# HYPNOTIC MODALITIES

## INTRODUCTION

As therapists, it is important that we recognize the different forms of Hypnosis that exist in society. The prime reason for this recognition is to understand the different elements which influence this process. For professional hypnotherapists, the efficacy of their treatment gets directly affected by the degree of control that they have on these elements.

## DEFINITION OF HYPNOTIC MODALITY

For the purposes of this class we will define hypnotic modality as:
Anything that attempts to control or modify human behaviour through the influence or creation of belief systems.

## THREE NECESSARY ELEMENTS

The effectiveness of a modality can be measured and even predicted by the presence and strength of the following three elements:-

### 1) AUTHORITY or ONE-UP-MANSHIP

"One-up-manship" is the process by which authority is established. This is necessary for the client's mind to be able to perceive the therapist as an authority figure and for the therapist to gain access into and beyond the critical area of the client's mind.

Another subtle method of establishing authority is controlling the environment in which the message is being presented. Many times a social struggle exists between the therapist and the client over control of the environment and the establishment of authority. The efficacy of the modality increases if the therapist maintains control.

One-up-man ship can be shown in various subtle ways, as under:
"Special clothes": These are used to socially separate the therapist from the client. For eg. When you go to see a doctor and if he shows up in jeans he may not have the same effect on you as he would if he had worn a suit with a lab coat.
"Space differentiation": Many orators use a podium and stage to maintain their position and hold attention. Or a doctors waiting room complete with receptionist et al.

"Discreet display": Most professionals display degrees, certificates, trophies and other such credentials on the office walls.

## 2) DOCTRINE/PARADIGM/TRANSLOGIC

This modality is essential to satisfy the critical area of the mind.

**Doctrine:**
It has three different forms of the same thing. Each one can be considered a lower level of the previous one. The first and highest level is the "doctrine". The doctrine is the written form of a theory or message. This usually comes in the form of a book or a printed manual.
By and large, doctrines are accepted by major sections of society as indisputable. Examples of doctrines are – the Bible, the Koran, the Gita etc. Many times the doctrine is written or inspired by an even higher authority than the presenter himself.

**Paradigm:**
A lower level than the "doctrine" is the "paradigm". A paradigm is a theory or model of how something works. The paradigm has not yet evolved into a book but rather is presented verbally or with charts and graphs.
Example: Theory of the Mind, Theory of Relativity etc.

Translogic:
Translogic is not quite as evolved as a paradigm or a doctrine. In fact, it is called translogic because it is transitory (temporary) logic. Also called "fast talk" it is transitory logic because it only makes sense for a very short period of time.
Examples: Salesmen, auctioneers etc.

The doctrine/paradigm/translogic used by the therapist must be made to fit the existing belief systems of the client, and not vice-versa.

## 3) OVERLOAD/INTERNAL EXPERIENCE

The purpose of this modality is to create an overload of message units so as to gain access to the reactive part of the mind (subconscious). If access to the reactive part of the mind is not gained, the messages given by the therapist will be dealt with using logic and reason, and will not affect the belief system of the client. If we are not able to reach and affect the belief system of the client, the message will have little or no effect on the client's behaviour. This element is called overload/internal experience because the purpose of the overload is to create an

Internal emotional and a physical experience within the client. The most important thing that could be said about this element is that the client must "FEEL IT". Once the client "feels it" he will believe it. How do we get the client to feel it? By creating an overload of message units, triggering an internal experience, and attributing that internal experience to the message being presented via the paradigm.

The above elements are synergistic and complement each other and hence, should be used to enhance efficacy of the therapeutic modality.

## CONCLUSIONS

The job of a professional hypnotherapist can be described as one of a professional belief system enhancer.

# CHAPTER 2
# ERICKSONIAN HYPNOSIS

## BACKGROUND

Milton Erickson (1901-1980) was a pioneer in the field of hypnotherapy. He developed techniques that are still in use today. Some of the techniques he developed were indirect, or inferred, inductions. He was both a Psychiatrist and a Psychologist. He was the founding President of the American Foundation for Hypnosis.

Erickson had polio when he was seventeen and was told that he would be totally disabled for the rest of his life. He spent the next year learning how to tell the position of his arms and legs, and through hypnosis (although he did not know it at the time) he was able to walk again, even to the point of being able to control his limp. At eighteen, he enrolled at the University of Wisconsin in both Medicine and Psychology. He took a seminar in hypnosis from Clark Hull, and became fascinated.

By the fourth year of college, he had gained a reputation as a Hypnotist. He did more to legitimise the field of hypnosis in the modern era than anyone else. He felt that the key to being good as a therapist was to be a good observer, learning the induction so well that you can watch the client rather than pay attention to what you are saying.

From all his life experiences, he developed his own approach to hypnosis, which came to be known as the indirect approach.

## INDIRECT APPROACH

In the indirect approach, the individual will respond better if they are given options and are not told what to do. The indirect responses work better if they feel they are making their own decision (inferred comments vs. literal ones).

Erickson developed a system of metaphors. In this system the client had something to do with what was happening. There was a need to utilise the individual's creative potential.

Erickson was a pioneer in the field of hypnosis for the deaf by using non-verbal methods of communication.

## INDIRECT FORMS OF SUGGESTIONS

## Indirect Associative Focusing

In this approach, the therapist raises a relevant topic without the client noticing it, without forcing it on the client.

**Example:** With your new attitude, you're going to feel on top of the world, just like Tiger Woods winning the golf tournament or Sachin Tendulkar scoring a century (as the case maybe).

Truisms Utilising Ideo dynamic Processes and Time

A truism is something no one is going to disagree with. This can also be thought of as finding a common ground with the client. After the truism, an ideo-dynamic (physiological) response is attached. This is the first step in rapport.

**Example:** You already know how to experience pleasant sensations, like the warmth of the sun on your skin. This is similar to the questions asked at the beginning of an arm raising induction, which elicit the 'yes' response.

Questions that Focus, Suggest, and Reinforce

In forming suggestions, you can ask questions that get you into the area that you need to explore with your client. These questions trigger a client to work on an issue.

**Example:** Would you like to find a spot you can look at comfortably? Do yo ur eyes get tired and begin to blink? It is a good idea when doing this to get an observable response (e.g. When you feel this, nod your head).

## Implication

This is the use of implied rather than direct (inferred vs. literal) responses. This can be accomplished by means of inflection (change in pitch) and pauses.

**Example:** There may be tendency…(pause)…for your arm to feel lighter and lighter, and start rising off the table.

## Therapeutic Binds and Double Binds

These binds and double binds create the illusion of choice.

**Example:** Would you like to experience a light, medium, or deep trance? Notice that the options are what level trance, not whether or not there will be a trance. Another similar example of this is: Would you like to go into trance now, or in a few minutes?

Compound Suggestions: Yes Set, Reinforcement, Shock, Surprise

This is a suggestion that is connected with a second suggestion. The first suggestion is a truism. Once it is accepted, the second and important suggestion is also accepted.

**Example:** We've all had the experience of the sun on our skin, and you can feel that warmth right now.

Contingent Associations and Associational Networks

This is very similar to a compound suggestion. It starts with something that the client is likely to do and finishes with something that the client is not willing to do.

**Example:** As you continue looking at the spot, your eyes will grow tired and close by themselves.

The Implied Directive

Time binding suggestion
Implied suggestion
Acceptance

**Example:** As soon as you hear the sound of my voice, only you and I will be here, and your right hand will descend to your thigh.

Open-Ended Suggestions

This is a big use of client creativity. It allows for a choice of any kind of response. It is a "your pick" situation.

**Example:** In the memory banks of your subconscious mind are all the memories of every lifetime that you have ever lived. As you enter the tunnel, your subconscious mind will pick a lifetime that is important for you to view today.

Cornering All Possibilities of Response

Confusion technique is used here. The therapist gives the client so much information that it is impossible not to respond.

**Example:** You may feel a hot sensation or a cold sensation; you may feel an itch or a tingling sensation, or numbness. Your arm may feel heavy or light.

**Opposition of Opposites**

Erickson likes to play with language. "You can forget to remember, or remember to forget." The subconscious does not like opposites. As a result, it becomes a double bind situation, and the client is more likely to respond to suggestions.

Disassociation and Cognitive Overloading

The idea here is to give the client so many things to think about and do that he/she is overloaded with message units, and goes into fight/flight.

Other Indirect Approaches of Hypnotic Forms

Erickson used all the techniques described above in many combinations. Sometimes there were multi-link chains of these.

**INVERSE META-MODEL PATTERNS**

Often the Milton model has been called the inverse Meta-Model. The Meta-Model is a set of language patterns that can be used to specify experience more fully. In contrast the Milton-Model provides the user with ways of being "artfully vague". Being artfully vague allows a communicator to make statements that sound specific and yet are general enough to be an adequate pace for the listener's experience.

The Meta-Model can be conveniently divided into three areas: A) Gathering information, B) Semantic Ill-formedness, and C) Limits of the Speaker's Model.

**Gathering Information**

As part of the Milton-Model this area is called Deleting Information, and is the most useful of the three areas for hypnotic purposes. The four sub-categories are:

**Nominalizations:** Nominalizations are words that take the place of a noun in a sentence, but they are not tangible, as they cannot be touched, felt, or heard. Words like *curiosity, hypnosis, learning, love,* etc. are nominalizations. They are

used as nouns, but they are actually process words. In the following example, the nominalizations are in italics: "I know that you have a certain difficulty *in* your *life* that you would like to bring to a satisfactory resolution…and I'm not sure exactly what personal resources you would find most useful in resolving this *difficulty*, but I do know that your unconscious mind is better able than you to search through your experience for exactly those resources".

**Unspecified Verbs:** No verb is completely specified, but verbs can be more or less specified. If a hypnotist uses relatively unspecified verbs, the listener is again forced to supply the meaning in order to understand the sentence. Words like *do, fix, solve, know, experience, understand, and remember,* are relatively unspecified. The sentence "I *think* this is true" is less specified than "I *feel* this is true".

**Unspecified Referential Index:** This means that the noun being talked about is not specified. For example in sentences like "*People can relax", "This can be easily learned",* or *"You can notice a certain sensation"* , the listener has the opportunity to apply the sentence to him or herself in order to understand it.

**Deletion:** This category refers to sentences in which a major noun phrase is completely missing. For example, *"I know you are curious"*. The object of the sentence is missing completely. Again, the listener can fill in the blanks with whatever is relevant in his/her experience.

Semantic Ill-formedness
**Casual Modelling, or Linkage:** Using words that imply a cause-effect relationship between something that is occurring and something the communicator wants - to occur; and hence invites the listener to respond as if one thing did indeed "cause" the other. There are three kinds of linkage, with varying degrees of strength.

The weakest kind of linkage makes use of conjunction to connect otherwise unrelated phenomena. For example, *"You are listening to the sound of my voice, and you can begin to relax."*
The second kind of linkage connects statements by establishing a connection in time. For example, *"When you hear the sound of my voice, you can begin to relax."*

The third and strongest linkage uses words actually stating causality. For example, *"You are listening to the sound of my voice, which causes you to relax."*

**Mind Reading:** Acting as if you know the internal experience of another person can be an effective tool to build the credibility of the hypnotist as long as the

"mind-reading" makes use of generalised language patterns. For example, *"You may be wondering what I'll say next."* Or *"You're curious about hypnosis"* .

**Lost Performative:** Evaluative statements in which the person making the statement is missing (lost) from the sentence are called Lost Performatives. For example, *"They think you're doing a great job".*

C) Limits of the Speaker's Model

Its two categories can be used to limit the listener's model in ways that produce trance, as well as other outcomes.

**Universal Quantifiers:** Universal quantifiers are words that indicate over-generalization, such as *all, every, always, never, nobody,* etc. For example, "And now you can go *all the way* into a trance".

**Modal Operators:** Modal operators are words that indicate lack of choice, such as *should, must, have to, can't, won't,* etc. For example, "Have you noticed that you *can't* open your eyes?"

ADDITIONAL MILTON-MODEL PATTERNS

**Presuppositions:** The way to determine what is presupposed and not open to question in a sentence is to negate the sentence and find out what is still true. The simplest kind of presupposition is existence

Presuppositions are the most powerful of language patterns, when used by a communicator who presupposes what he or she doesn't want to have questioned. Give the person lots of choices, and yet have all of the choices presuppose the response you want. Some examples of presuppositions that are useful in hypnotherapy are:

**Subordinate Clauses of Time:** Such clauses begin with words such as before, after, during, as, since, prior, when, while, etc. For example, "While you keep going deeper, I want you to focus all your attention on your breath."
**Awareness Predicates:** Words such as know, aware, realize, and notice can be used to presuppose the rest of the sentence. For example, "With each and every breath that you take, you become more aware of the deep relaxation that you feel."

**Adverbs and Adjectives:** Such words can be used to presuppose a major clause in a sentence. For example, *"Are you curious about your developing a trance*

*state?"* This presupposes that you are developing a trance state; the only question is whether you're curious about it or not.

**Change of Time Verbs and Adverbs:** Words such as begin, end, start, stop, continue, proceed, already, yet, or still, can be used effectively as presuppositions. For example, "Even though you may close your eyes, you will continue to see the image of your hand."

**Commentary Adjectives and Adverbs:** These are words such as fortunately, luckily, innocently, happily, or necessarily. For example, "Fortunately, there's no need for me to know the details of what you want in order for me to help you get it."

Stacking many kinds of presuppositions in the same sentence makes them particularly powerful. The more that is presupposed, the more difficult it is for the listener to unravel the sentence and question any one presupposition. The following is an example of the use of many presuppositions stacked together: *"And I don't know how soon you'll realize the amount your subconscious has already learned, because it's not important that you know before you've comfortably continued the process of relaxation and allowed the other you to learn something else of use and delight to you."*

**Indirect Elicitation Patterns:** This group of patterns is particularly useful in getting specific responses indirectly, without overtly asking for them.

**Embedded Commands:** Rather than give instructions directly, the hypnotist can embed directives within a larger sentence structure. For example, "You can begin to *relax*" or "I don't know how soon you'll *feel better*." When you embed directives within a larger sentence, you can deliver them more smoothly and gracefully, and the listener will not consciously realize that directives have been given.

**Embedded Questions:** Questions, like commands, can be embedded within a larger sentence structure. For example, *"I'm curious to know what you would like to gain from hypnosis."*

**Negative Commands:** When a command is given in its negative form, the positive instruction is generally what is responded to. For example, if someone says, "*Don't* think of pink polka dots" you have to think of pink polka dots to understand the sentence.

# CHAPTER 3
# BODY SYNDROMES

Body syndromes are a very effective part of Hypno-diagnostic tools.

When somebody holds an emotion in, rather than releasing it, it will manifest itself physiologically and the person will feel physical discomfort.
The body will suppress the emotional problem and hold it in, finally manifesting itself in the area of the problem (body syndrome). There are five syndromes:

## A) Crying Syndrome

The area covered is from the solar plexus up to and including the neck and head. This will manifest itself to indicate the inability to make a decision that the client is not happy with.

It is called the crying syndrome because when someone is in this state, one's eyes crystallize. Some physiological ways in which this shows itself: Asthma, arthritis, and sore throat, anything concerning hearing, the eyes or the sinus.

## B) Responsibility Syndrome

The involved area here is the shoulders and upper back, including the upper spine. This is the area of responsibility, hence the name. The term "Carrying the world on your shoulders" may have come from this.

## C) Sexual Frustration or Guilt Syndrome

The area of most involvement is the groin, hip, or lower back areas. This area indicates the sexual frustration or possibly guilt about something the client does not care to deal with. Possible problems are lower back pain, or problems in the lower GI tract.

## D) Fight Syndrome

The areas here are the arms and hands. The client may be suppressing the need to fight or are constantly trying to reach out to goals which are unrealistic.

### E) Flight Syndrome

The involved areas here are the lower extremities. Here the client is running away from a problem.

**Note:** There may be more than one syndrome involved at any one time.

## DEALING WITH WEIGHT

The therapist can use body syndromes to find out what kind of weight problem he is dealing with. Have the client grade their body in each of these five areas. The low scores would likely be a body syndrome, and indicate that the weight might be linked to another problem. If the grades are equal or close, the problem is probably a habit pattern.

# CHAPTER 4

## EMOTION FREEDOM TECHNIQUE

Emotional Freedom Technique also known as EFT has based created by Gary Craig and is based on the principle that the reason behind all the emotional disturbances one experiences is a disturbed energy system.
We can reverse this disturbance by artificially creating a fresh disturbance. In an attempt to overcome the artificially created disturbance, the body vitality sets the system right.

**PRINCIPLE:**
Thoughts, emotions, energy levels are interlinked & are known to influence the body & vice versa.
There is close association between emotions & energy. Enthusiasm brings in a zip & zing in the energy system. On the contrary, despair saps the energy out of the system. Thus, any emotional disturbance is accompanied by a corresponding distortion of the energy lines. Once emotional stability is attained, the energy lines revert back to their normal state & vice versa.

**The Concept of Reversibility & Law of 'Similars',**
We can reverse any energy disturbance created by a disturbed emotional state by inducing a similar fresh external artificial disturbance at the level of energy. This is done by gently stimulating certain energy points. In an attempt to overcome the artificially created disturbance, the person's vitality sets the system right & the corresponding emotions get balanced.
**When Is EFT Useful?**
Disturbing emotions like fear, anger, worry, tension, anxiety, depression, hatred, jealousy, insult, frustration etc.
Physical aches & pains.
Understanding the core emotion.

**TECHNIQUE:**
**[STEP 1]**
Pay attention to the problem in your body or mind (Example: Pain or anger etc).

**[STEP 2]**
Get in touch with the emotions/feelings associated with the complaint/problem.
(Example: 'Headache after feeling upset following an argument' OR 'Angry when things don't go your way')
Try and be specific in assessing the feeling. The clearer the feeling becomes, the quicker one finds relief. Nevertheless, the technique works but may take a bit longer.

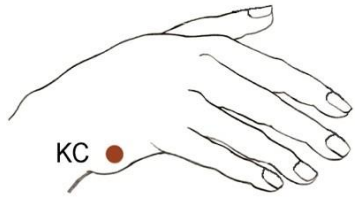
**[STEP 3]**
Now give this emotion or complaint a number on scale of 0 - 10, depending on the intensity & severity, zero being no pain or complaint & 10 being the maximum in severity. i.e. subjective unit of discomfort – SUD score
(You may write down the feeling/complaint with the grade in front of it, for your reference as you work on it further.)

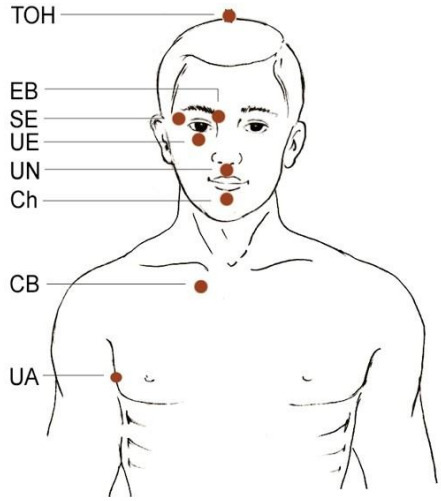
**[STEP 4]** Making the affirmation phrase:

When designing this phrase, we focus on two important aspects:
-Acknowledge the problem
-Accept yourself in spite of the problem

"Even though I feel/have..... (the problem or issue), I deeply and completely accept myself and choose to heal and integrate that part of me."
Examples:
(a) Even though I have *this fear of exams*, ……choose to heal & integrate that part of me.
(b) Even though I have *this fear of failure*, ….
(c) Even though I have *these nightmares*, …..
(d) Even though I have *this headache*, …..
(e) Even though I have *this stiffness in my neck*, …..
(f) Even though I have this craving for junk food/chocolates,….
(g) Even though I am feeling very upset as my teacher scolded me,….
(h) Even though I am feeling angry towards my mother, ….
(i) Even though I *don't like XYZ subject*, …..still I choose to heal & integrate that part of me.

**[STEP 5]** Say the affirmation ***Three Times***, *loudly with emphasis*, while gently rubbing/tapping the following points:
Diagram 1] The karate chop (KC) point in the body, on the sides of the palms:
Diagram 2] Now only repeat the key words from the phrase i.e. this fear, this headache, this pain, this anger, etc
-Top of the Head (TOH)
- Beginning of the Eyebrow (EB)
- Side of the Eye (SE)



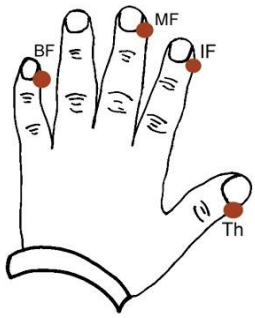




DIAGRAM 1 DIAGRAM 2 DIAGRAM 3

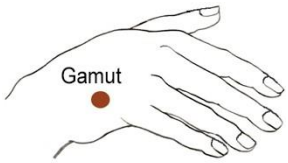



DIAGRAM 4

(diagrams ref. www.emofree.com)

- - Under the Eye (UE)
  - Under the Nose (UN)
  - Chin Point (CH)
  - Beginning of the Collarbone (CB)
  - UndertheArm(UA)

Diagram 3] Tips of the fingers

Diagram 4] While facing forward and keeping your head still, tap the Gamut point continuously while performing the following nine actions:

- Eyesclosed
- Eyesopen
- Eyesharddownrightwhileholdingtheheadsteady
- Eyesharddownleftwhileholdingtheheadsteady
- Rollyoureyesinacirclecl ockwiseandanti -clockwise
- Hum2secondsofasong(eg.Happybirthday – aseveryoneknowsit)
- Countrapidlyfrom1to5
- Humthesongagainfor2seconds

**[STEP 6]** Then, gently place your palms on the solar plexus and take seven deep breaths – breathing in through the nose and blowing out slowly through your mouth.

**[STEP 7]** Check the intensity of the problem.

If the problem is *relieved totally* i.e. come down to '0' on the scale of 0-10, then think of any pleasurable event or person and connect to the feeling of joy. Then, go on to the next emotion/feeling that may crop up.

If the problem is *relieved partially*, then mentally give it a number between 0 - 10, & start another round of EFT saying the slightly modified affirmation i.e.***"Even though I have this remaining…… I deeply and completely accept myself and choose to heal and integrate that part of me."*** (Example: "Even though I have this remaining headache, OR "Even though I have this remaining anger,)

If there is no change at all, your subconscious mind is probably resisting the change. In such a situation do the EFT by saying **"Even though I don't want to get over…… I deeply and completely accept myself and choose to heal and integrate that part of me."** (Example: "Even though I don't want to get over my headache, OR "Even though I don't want to get over my anger)

NOTE:
* Say the affirmation loudly with feeling & emphasis rather than saying it mechanically.

* EFT can be done anywhere, anytime & any number of times till there are no perceived issues or problems and there is a spontaneous change in the thought process.

** EFT can be used before, during or after the therapy.*

* Yawning, sighing, coughing, passing gas are very commonly seen signs & indicate that this technique is working effectively.

*If you or your client finds a particular point gives more relief during tapping, you can choose to stick to that point and complete the EFT process.

*Breathing from the solar plexus gives the client charge of his/her life.

## EMOTION MANAGEMENT TECHNIQUE

**TECHNIQUE: similar to EFT.**
**[STEP 1]**
Pay attention to the problem in your body or mind (Example: Pain or anger etc).

**[STEP 2]**
Get in touch with the emotions/feelings associated with the complaint/problem. (Example: 'Headache after feeling upset following an argument' OR 'Angry when things don't go your way'). Try and be specific in assessing the feeling. The clearer the feeling becomes, the quicker one finds relief. Nevertheless, the technique works but may take a bit longer.

**[STEP 3]**
Now give this emotion or complaint a number on scale of 0 - 10, depending on the intensity & severity, zero being no pain or complaint & 10 being the maximum in severity. i.e. subjective unit of discomfort – SUD score

**[STEP 4]**
"Even though I feel/have….....(the problem or issue), I deeply and completely accept myself and choose to heal and integrate that part of me."

**[STEP 5]** Say the affirmation ***Three Times***, *loudly with emphasis*, while gently rubbing/tapping the center of the palm of one hand with the thumb of the other hand.

**[STEP 6]** Then, gently place your palms on point GV20 i.e. center of the crown for the count of seven breaths.

**[STEP 7]** Check the intensity of the problem and use one of the following as required:

- ***Even though I have this remaining…….*** (Example: "Even though I have this remaining headache, I choose to heal & integrate that part of me." OR "Even though I have this remaining anger, I choose to heal & integrate that part of me.")
- ***"Even though I don't want to get over …… ……I choose to heal & integrate that part of me."***(Example: "Even though I don't want to get over my headache, I choose to heal & integrate that part of me." OR "Even though I don't want to get over my anger, heal & integrate that part of me.")

## GUIDELINES FOR CONDUCTING A HYPNO-DRAMA

- Conduct a very effective pre-induction interview, try and get to know all facets related to the client's problem in the relationship.
- Cognitively ask the client – that once he/she heals out of the toxicity surrounding the relationship, what would the client like to experience/feel about it? Is there any real life incident in the client's memory in which he has experienced this particular positive feeling before? Ask the client for an appropriate trigger.
- Induce as deep a hypnotic state as possible.
- Take the client back in time to the desired good memory. Bring up the appropriate positive feelings let the client psychologically and physiologically experience this completely. Associate with the trigger.
- Gradually, let this feeling recede into the background for the client.
- Using guided imagery, let the client visualize that he is in a room of his choice with two chairs. He is sitting on one of the chairs (alternatively, the client maybe encouraged to utilize any environment of his choice – suited to the event).
- Let the person with whom he is having a problem come into the room/picture and sit on the other chair.
- For a client under hypnosis, this scene is very real. Let the client bring up all the pent up emotions that he has for the other person. Give the client ample time to do so.
- Gently encourage the client to vent out totally and completely all the negative thoughts, emotions and energies that the client has associated with the other person. The client at this stage may exhibit physiological symptoms.
- Confirm with the client, that he has effectively released all the stored emotions.
- Shift the client's consciousness to the good feelings that he had brought up in the beginning.
- Guide the client to use the processing skills of his conscious mind and transfer this good feeling into the vacuum/void created in his mind by the exit of the old toxic feelings. Use the trigger.
- Test the validation of the therapy with an imaginary situati on in which the client interacts with the other person, now, in a very appropriate manner, activating the trigger as required.

# CHAPTER 5
# DREAM THERAPY

## INTRODUCTION

Dream therapy is one of the most popularly used modalities in clinical practice for several reasons, as under:

**First**, it augments the venting process of hypnosis.
**Secondly**, since dreams occur during the relatively safe passage of sleep for the client, its investigation / probing by the therapist becomes a non-invasive and harmless process to get an insight into the inner workings of the client's sub-conscious mind without the risk of age regression.
**Lastly**, the most significant impact of dream therapy is that it serves as an excellent feed-back tool for the therapist about the progress that the client is making during the therapy. Depending on the content of what is being vented out through dreams the therapist can determine the efficacy of his line of action.

## NO EXPERTS

The first rule of dream therapy is that there are no "experts" in dream interpretation, and there is no instant analysis of dreams. Instead, the dream analysis therapy is a process that the client and the therapist do together. We believe that it is the therapist's job to make the client as much an expert in the process as the therapist is. The therapist does not interpret dreams "for" the clients, but rather assists them in interpreting their own dreams.

When therapists start feeling that they are supposed to come up with some stupendous analysis of the dreams, they begin avoiding dream therapy. Consequently, they deny their clients perhaps one of the most therapeutic aspects of hypnotherapy.

## DREAM SYMBOLS

Many people have gone to their local bookstore to look for a book on dream symbols, only to find material that resembles a dream "dictionary". These dream dictionaries list the meanings of dream symbols. Many of these definitions are very psycho-analytical and would suggest that the symbol means the same thing for all people. This suggests that dream symbols are "Static" or never changing.

We assume that dreams are not "Static" but rather "Dynamic", always changing and different for each person. With dynamic dream symbols, there can be no

experts because these mean something different for each client. The therapist's job then is not to know automatically what the dream symbol means, but rather to know what questions to ask the dreamer so that the dream symbols can be translated into the language that is obvious to all.

LANGUAGE OF DREAMS

Dreams emanate from the creative side of our brain which is the right side. Unfortunately, we interpret these dreams using the left side of our brain which operates on the paradigm of logic and reasoning. As a result, we very often tend to dismiss dreams as irrelevant or illogical, simply because we don't have the requisite processing skill to be able to understand them.

MNEMONIC LEARNING DEVICE

Some People Teach Children Entirely Via Television.
The preceding sentence is a mnemonic learning device that will help you to remember the seven questions that the therapist must ask the client BEFORE attempting an interpretation of the dream. These questions are listed in the Dream Therapy Worksheet in this chapter. Note that the first letter of each word in the mnemonic learning sentence corresponds with the major concept in each of the seven questions that the therapist must ask. By remembering this simple sentence you will always be able to remember what questions to ask your client during dream therapy.

TONIC MOTOR INHIBITION

Tonic motor inhibition is the process whereby your mind cuts off the electrical impulses to the motor muscle groups during dreaming. This is what prevents you the dreamer from thrashing about or moving your arms and legs during a dream. This is a protection mechanism so the dreamer can experience venting all sorts of emotionally latent situations without risk of injury to self.

STIMULUS INCORPORATION

When the dreamer experiences a physiological stimulus during deep sleep, that physiological stimulus becomes automatically incorporated into the content of the dream. This process is referred to as stimulus incorporation and is the basis for physiological dreams. Because the content of a physiological dream is the result of a physical stimulus, it would be incorrect to make psychological interpretations of physiological dreams.

## DREAM SUGGESTIONS

It is a good idea to reinforce dream therapy each and every time you hypnotize your client. A sample suggestion that could accompany the post-hypnotic suggestion for re-hypnosis, both when the client first goes into hypnosis and again when you are bringing him out, is:

Each time I suggest deep sleep, you will sleep deeply, soundly, and quickly, to this depth or deeper…and the hypnotic sleep like your dream sleep is a time for your mind and body to vent out any thoughts, feelings, or ideas that are no longer beneficial to you. A mental emotional cleaning house, and as you vent out all those unwanted message units, your tolerance for stress grows stronger. Each time that you remember a venting dream, just like each time you experience hypnosis, the conditioned process grows stronger and more effective.

Another good dream suggestion is to give the client hypnotic suggestions in the first appointment to have venting dreams.

You are going to experience a series of dreams this week. These dreams are going to be venting dreams. They are going to be very vivid, and you are going to be much more likely to wake up remembering these vivid dreams. When you wake up remembering these dreams it is going to remind you that the hypnosis is working and that change is beginning to take place now.

These can be very powerful suggestions in the first session for several reasons. One is that just by hypnotizing them for the first time and triggering that process of REM activity, you are in effect stimulating them to dream that week. This suggestion works similarly to the suggestion of "draw your attention to your breathing and notice it change." They are going to dream anyway. Drawing their attention to it creates the illusion of change. The purpose of this type of suggestion is to plant the idea in the belief system of the client that hypnosis does work. "The hypnotic suggestions do affect my brain."

After you give the client this suggestion the client will tend to forget about it, until a few days later when they wake up remembering their dreams. It is then that the post-hypnotic suggestion has the impact on the client of "Oh my god, it is working. That hypnotist really had an effect on my mind." This is a very powerful and a very desirable effect on the client and their response to all future suggestions.

## DREAM INTERPRETATION WORKSHEET

1. What **STAGE** during the night did the dream occur?
2. Wishful Thinking Stage
3. Precognitive Stage
4. Venting Stage
5. Was the dream PSYCHOLOGICAL or PHYSIOLOGICAL?
6. What was the **TIMING CLUE** in the dream?
7. What is the **LITERAL CONTENT**? What is the **SYMBOLIC CONTENT**? Define the symbols.
8. What are the **EMOTIONAL FEELINGS** of the dream?
9. What is being **VENTED** out?
10. What **TRIGGERED** the dream? How does the dream **RELATE** to your present life?

# CHAPTER 6
# STAGES OF DEVELOPMENT

All stages explained herein are those defined by Eric Erickson. People develop according to patterns, but the ages may vary somewhat according to the individual. Erickson's approach is from a behavioral aspect while Freud's is psychoanalytical.

If a client continues to regress to a particular stage, that stage was never completed. In therapy, the client is taken back to that stage so that it may be completed.

**Stage One: INFANCY**

Ages 0-2

Tries to fulfill needs, Learns to walk and talk, If needs are met, infant develops a sense of basic trust.

It is during this stage that the mind is most receptive. The mind, at this point, is like a sponge and soaks up whatever it needs. For example, a child who cannot speak may have a vocabulary of up to 15,000 words. Information is very precise and unambiguous.

Radius of interaction: Maternal

Potential Strengths: Hope and drive. The genetic drive to develop is very important. The child is an optimist.

Psychosocial Development: Trust Vs mistrust.

**Stage Two: EARLY CHILDHOOD**

Ages 3-5

Starts to prove self, Wants to do things by self

Suggestibility formed initially from primary caretaker (usually mother).

Toddler strives to learn independence and self -confidence

This is the stage when we want to do things for ourselves (e.g. tie shoes, get own toys etc.). At this time we begin to assert our individuality. We begin to develop our suggestibility based on our primary caretaker.

Potential Strengths: Self-control and willpower. This is the stage where we begin to learn self-control.

Psychosocial Development: Autonomy vs. Shame and Doubt

**Stage Three: PLAY AGE**

Ages 6-9

Attachment from parents to self

Added suggestibility from peer group
Need to prove self in school, Preschooler learns to initiate tasks and grapples with self-control

This is the initiative stage, when the child wants to go out on his own and discover what he can do. The child becomes less dependent on parents and more dependent on himself. Passive Aggressive behavior may begin to develop.

Passive Aggressive Behavior: The child will go out of his way to sabotage the parents. As an adult, he may sabotage his career, etc. He tries to do things that will hurt him. This may develop when the child is punished for showing initiative. Radius of Interaction: Basic Family.

Potential Strengths: Direction and purpose. This is the stage where the child generally decides what he wants to be when he grows up.
Psychosocial Development: Initiative versus guilt.

**Stage Four: SCHOOL AGE**

Ages 10-13
Closeness to father and boyfriends (males only)
Closeness to mother and girlfriends (females only)
Sexuality forming
Homosexuality trends
Religious guild
Rather be with friends than with family
Child leans either to feel effective or inadequate

This stage and stage five are generally the most critical to therapy. This is the stage where the child has the greatest potential to receive mixed messages, creating a lot of confusion. These messages may lead to homosexuality. Double messages from parents may help to create schizoid behavior. Also, good or poor study habits may develop at this time.

Radius of Interaction: School and neighborhood.
Potential Strengths: Method and competency will develop in a healthy child.
Industry: (Competency) Industry is our ability to follow through with anything; inferiority may result when we cannot. This stage usually is a period of steady growth. We become more sensitive to how others perceive us. When a child makes a real achievement, this is industry; when the child fails, a feeling of inferiority is the result.

Sexuality: Industry reinforces the physically sexual, while inferiority reinforces the emotionally sexual. Inferiority may develop if there is either too much or too little expected of the child.
Psychosocial Development: Industry versus Inferiority.

**Stage Five: PUBERTY AND ADOLESCENCE**

Ages 14-21
Some conflict with mother
Some conflict with brothers and sisters
Search for opposite sex
Much emotional pressure
Sexuality formed from father
Wants father's approval
Most receptive to hyper-suggestibility
Most receptive to marriage
Stage to which client will most commonly regress
Stage in which greatest degree of guilt may be experienced

People often revert to this stage. The problem is identity versus identity diffusion. Identity is not solid during adolescence. The person is now faced with who he/she is becoming. The adolescent sometimes acts as a child and sometimes as an adult. Physiological changes are also occurring (e.g. puberty).

The adolescent is faced with the idea of being on his own in a few years. The primary struggle is one of ending childhood and facing the future. The greatest problem here is one of diffusion. He may try to be too many things. The emotional sexual may diffuse himself (spread himself too thin), while the physical sexual will usually not. The emotional sexual can get confused at this time, especially if the father is non-verbal and doesn't give emotional approval. This will solidify emotional sexuality. A physical sexual father will solidify physical sexuality.
Radius of Interaction: Peer groups.
Potential Strengths: Devotion and fidelity.
Psychosocial Development: Identity versus confusion (the identity is diffused).

**Stage Six: YOUNG ADULTHOOD**

Ages 22-30
From "me"-ness to "we"-ness
Own family and children

May start therapy if single or divorced
Tendency to have doubts about self or lifestyle
Feels closer to opposite sex
Feels closer to own sex if stage four has not been completed
Tendency to cheat in relationship if stage five is in conflict

This stage may last as long as age 40. This is where the "Seven Year Itch" may develop. There may be a tendency to cheat in a relationship. The young adult who is troubled is more likely to come into therapy at this time. "Me"-ness is developed in the emotional; "We"-ness is developed in the physical.

Radius of Interaction: Partner in relationship. This is usually when sexual competitiveness is learned. The partner in the relationship may aid in helping career.
Potential Strengths: Affiliation and love.
Psychosocial Development: Emotional and Physical behavior may be more pronounced. The physical develops intimacy while the emotional develops isolation. **Intimacy vs. Isolation**

**Stage Seven: MIDDLE AGE**
Ages 31-40
Responsibility for others and work
Wants independence
High motivation
High potential of going into therapy
If single, urgency about having a relationship
If previously married, guilt of passing stage six
Most receptive to physical problems if stress in stage five

Generativity is being more outward and expressive. If not, the opposite is to pull all the thoughts and feelings inward.
The mature adult who has achieved intimacy is connected with the future generations in addition to himself.

People want to make career changes. If a person is still single, there is urgency about developing a long-term relationship.

Radius of Interaction: Work, career, and family.
Psychosocial Development: Generativity versus self-absorption.

**Stage Eight: LATER LIFE**

Ages 41-50
Needs understanding of life
Needs to be stable
Fear of old age
Make last attempt at what has been missing in stage five
May develop areas of doubt about self and others
Attempts to stabilize lifestyle
May need to improve communication
May need to socialize with the same age groups

At this time, a fear of death and dying may develop. The person may be faced with aged parents. Sometimes he is responsible for the care of the aged. Commitment to a spouse may become very important.
Radius of Interaction: "My people" or Mankind. The person becomes part of society. He has his life experience behind him. This is the time to evaluate whether or not life has had any meaning.
Potential Strengths: Wisdom versus renunciation. It is time to accept the way you are, if possible.
Psychosocial Development: Integrity versus despair.

When this stage is reached, you either accept yourself or you don't. Despair may be characterized as having too much attachment to family and friends.

# CHAPTER 7
# CORRECTIVE THERAPY

This is done cognitively. Have the client write down the problem in sentence form.

If the client is a physical suggestible, have them go directly down the columns, giving another word for the word at the top of the column.

If the client is an emotional suggestible, use the same process but use the previous word instead of the one at the top of the column.

**Example:** The word "I" – with a physical, have them give you different words for "I". With an emotional, have them give you a word for "I", e.g. "me", and then another word for that word, "me", and so on.

Do this until it becomes difficult for the client (about five times). The last line will tell the therapist what the client feels about a problem. The last line is the subconscious problem.

Note: This cognitive questioning for replacing the words should be done at a fairly rapid speed which facilitates the response coming straight from the subconscious mind.

# CHAPTER 8
# HYPNO-AIDS

Age and attention span may necessitate the use of hypno-aids. They are used to get attention and lengthen the attention span. They also may be used with a client who has preconceived ideas about hypnosis.

- **Crystal Ball:** This is normally used only with children.
- **Pendulum:** Traditionally most commonly associated with hypnosis.
- **Penlight:** Never use this with a client that has enlarged or thyroid-type eyes. When working with a client like that, never use an eye challenge.
- **Metronome:** This is good to use with ear-minded persons. Best if set at 60 beats/ min.
- **Air machine:** This produces "white sound", or sounds similar to an ocean breeze. This is lulling and can aid the hypnotic effect.
- **Hypno-disk:** This works by creating an overload of visual message units.

## IDEOMOTOR RESPONSES

The therapist uses this when he wants to get a reply directly from the central nervous system. It is a way of avoiding the judgment of the conscious mind, and of bypassing the critical mind. This can be used either in or out of hypnosis.

The client must concentrate on the question, not the answer. The answer will come from the subconscious mind. It occurs when the thought of movement produces the answer. The question must be a yes or no question. Small muscles will elicit the best response (e.g. finger signals). Explain to the client that you are going to ask a series of questions that will go directly to the subconscious mind. Mutually select two fingers and decide that one finger will be the 'yes' response; the other will be the 'no' response. Flood the mind with the question.

Another form of this is to use the crystal with yes or no questions. The crystal works best because the answer is really coming from the central nervous system, and will cause you to move the crystal subconsciously. Once it is determined what the yes and no responses are, the therapist can ask questions and get the answers directly from the subconscious mind.

# CHAPTER 9
# PARIS WINDOW

The Paris Window consists of four squares. The first three squares are questions; the fourth is the solution. Ask the client the following (not necessarily during the same session):

1. How do you feel about the problem?
2. How do you think others feel about your problem?
3. How do you feel about how others feel about your problem?
4. Resolution.

Now, in the fourth square of the window, come up with the answer to what the problem is. This gives us as therapists a good insight to find out where we are going. The client can only see from the surface. They can't see from the subconscious level.

# CHAPTER 10
# FIVE STAGES OF LOSS

There are five stages that a client must go through to completely deal with a traumatic loss, such as death, divorce, etc.

**1) Denial**
Refusing to accept that the loss has occurred. This stage can last for years.

**2) Anger**
This may be directed at either the person who has left or at anyone else who happens to be there. In some cases the anger is directed at the therapist.

**3) Depression**
This stage and anger go hand in hand. The person will keep going back and forth between these two stages until it is time to go on to the next stage.

**4) Bargaining**
The client will bargain with anyone: God, the therapist, anyone in his or her life. They will even bargain with themselves.

**5) Resolution**
This is the final acceptance, and the beginning of putting one's life back in order. In therapy, take the client through all five stages. Explain the stages, so they will understand what they are feeling.

This may be used to deal with loss due to death, divorce, etc. It can also be a valid way to treat a terminally ill client.

# CHAPTER 11
# UNDERSTANDING DEPRESSION

The material given here is very brief, you may have to study in greater detail, if you have to handle a client.

**Symptoms of depression:**

1. **Minor depression**: Loss of energy, motivation and appetite; but they can still function normally and get their necessary things done.
2. **Major depression**: Any 5 of the following symptoms, felt continuously for 2 weeks or more, may indicate major depression.
    a. Depressed mood most of the day, nearly every day.
    b. Diminished interest or pleasure in almost all activities of the day, nearly every day.
    c. Significant weight gain or loss when not dieting, and decreased appetite nearly every day.
    d. Insomnia or hypersonic (sleeping too much) nearly every day.
    e. Abnormal restlessness or a drop in physical activity nearly every day.
    f. Fatigue or loss of energy nearly every day.
    g. Feelings or worthlessness or excessive or inappropriate guilt nearly every day.
    h. Diminished ability to think, concentrate, or make decisions nearly every day.
    i. Recurrent thoughts of death, or suicidal thoughts or attempts.

Symptoms of depression: These could be emotional, physical and psychological. They can be experienced individually or simultaneously.

**Triggers for depression**

1. Stress
2. Traumatic events
3. Genetic factors and personality types.
4. Specific diseases and major illnesses
5. Medications
6. Alcohol or drug abuse
7. Hormonal imbalance
8. Chronic patterns of negative thinking.

# CHAPTER 12
# HYPNODRAMA

## PSYCHODRAMA

Most of our issues stem from conflicts stored in our sub-conscious mind. These conflicts often become the triggering factor behind our repetitive behavioral patterns. Psychodrama is a process by which these sub-conscious conflicts are acted out in a relatively safe environment for the client, thereby providing him an opportunity to vent and resolve the issue. The hypnotherapeutic equivalent is hypnodrama which has several advantages over psychodrama.

## PARATAXIC DISTORTION

A parataxic distortion is when we respond to a person or situation in a distorted way. We are not responding to the situation or person, but rather to what they subconsciously trigger in us. We may have an unconscious conflict with a person because of some unresolved issue. Therapists would look for these parataxic distortions and then attempt to resolve them by recreating them.

## MICROCOSM

A microcosm is a small part that is representative of the larger whole. The way a client behaves in the therapeutic setting can be looked at as a microcosm of the world. What takes place there often represents what goes on in the real world. Within a microcosm a client normally demonstrates his/her own true behavior. If the person reacts to a person in a group setting with a parataxic distortion, they are more than likely also doing that out in the environment. The therapist wants these distortions to show up in therapy so that they can work them out.

## THE EMPTY CHAIR TECHNIQUE

One of the best known Gestalt therapeutic techniques is the empty chair technique. In this process the therapist has the client deal with the parataxic distortion by imagining that the person they have a conflict with is sitting in the empty chair. They are then encouraged to get their feelings out by verbalizing them to the imagined person. They then take the role of the person in the chair. Through this process they role-play the conflict. This process is similar to circle therapy in that you try to trigger the parataxic distortion and vent out the negative energy. Emotions come in through the senses, as stored as electro-chemical energy, and are released through the senses. They must feel it to release it.

A psychodrama is effective if they experience strong feelings and are able to release the energy. The therapist plays the role of the director, getting them to act it out. If they are afraid of feeling or expressing their emotions, they probably will not be able to let go of this conflict.

HYPNODRAMA

In hypnodrama the goal is the same, but there are several advantages over psychodrama. For one, there is less embarrassment in having to act these feelings out. Another advantage is that in hypnosis there is more access to the emotions and senses. When the drama takes place in the mind, we can also change the realities of the drama. If we were going to create the ideal psychodrama, we would recreate the exact sensory experience. The more senses we tap into, the more we are able to re-experience the conflict and thereby release the energy tied into it.

NEURO-PATHWA YS

Every time we think a thought, make a movement, or experience something, this is transformed into electro-chemical energy, which is then stored in the brain. We create pathways that allow the energy to travel in a similar fashion each time it is triggered. The more it is triggered, the easier it is for the energy to go that route. This is how habits are created. Energy always takes the path of least resistance. If the original path leads to a negative result and keeps getting reinforced, it will become increasingly difficult to redirect the energy to a positive result. Every time you face the situation, the energy will take the same well-traveled path. To prevent this, we must create a new path that will lead to better results. As we direct the energy towards a new path, the old path gets weaker. By rehearsing this new behavior in hypnosis, we create a new path.

USING HYPNODRAMA

Find out what the person's parataxic distortions are in the interview. Find out all the senses and memories of the situation. In hypnosis, have them get in touch with as many senses as possible. Have them rehearse it with intensity. The more intense it is, the more powerful the subconscious impression will be. Identify the distortion and change it. Make the distortion worse to the point of being ridiculous. Then take it to the opposite extreme. When you reframe the distortion, you take its power away. Get the client to future pace. Have them see it the way they see it now; then have them reframe the distortion; finally have them see themselves in the future with a more useful behavior.

It takes imagination to create a hypnodrama. You must be a good director and use your voice and emotions to maximize their experience. If the distortion is traumatic for the client, have them see it on a TV screen or have an actor play their role.

SPORTS

You can also use hypnodrama to improve sports performance. Rehearsal, in the end, can improve performance tremendously. Have them describe their sensory impressions of the experience so you can incorporate these into the hypnodrama. The more senses you incorporate the more powerful their experience. Find out if they have parataxic distortions to reframe. If they are Emotionals, go for the feelings and the end result. If they are Physical, run through the whole event being literal and direct.

DEFINITION OF HYPNODRAMA
A behavior modification process used in hypnotherapy, where the therapist guides the client in hypnosis, through an actual event or life experience specified by the client.

The therapist encourages the client to feel the experience with all the sensory, emotional and physical sensations associated with the event. The therapist gives the client adequate time to stay in the experience, and to allow the subconscious to process the event as if it were “real”.

GUIDED IMAGERY

The therapist uses an imagery script designed to elicit specific responses and experiences.

For example, with cancer patients a guided imagery script might suggest, “You make yourself very small and go inside your body, entering through your nose or mouth. You can be in a capsule or bubble to protect yourself. You are safe and you can breathe easily. Enter your bloodstream now, and notice your white cells. Notice how powerful they are, how efficient…”

GOALS OF HYPNODRAMA

1. Tailor the hypnodrama to match the client’s experience.

1. Provide a mechanism to vent out the negative emotions associated with the experience.

a. Do this in hypnosis by exaggerating the emotions and then suggesting that the client releases or reframes them.

b. Do it cognitively in your discussion with the client – encourage them to express the painful aspects of the problem.

c. Clients must FEEL the difficult emotions before they can release them.

d. They won't be able to make the shift to imagine an ideal outcome without venting the painful parts first.

2. Get into your client's 'microcosm' or world as intensely as you can:
   a. Ask lots of questions to uncover the client's perception of the desired end state.
   b. Use your client's keywords exactly.
   c. Allow room in the hypnodrama for the client to build experiences that are meaningful for them personally.

3. Use plenty of anchors.

**ANCHOR** is a word, symbol, physical act, or any stimulus in the environment that the hypnotherapist associates with desirable body-mind responses. The anchor activates the desirable state, reinforces it repeatedly, or can be used by the client at will to energize desirable behavior.

Examples of anchors:

- Every time you notice the color blue, you will be aware of a sense of well-being radiating throughout your being.
- Whenever you see someone wearing surgical scrubs, you will notice your sense of calm deepening, and you'll be reminded of all the things you plan to do when you recover.
- Whenever you pick up your pencil, you will notice a sense of calm radiating from your hand into your entire being.

4. Engage the senses – MAKE IT REAL! MAKE THEM FEEL IT!
   - Sight
   - Sound
   - Touch and texture
   - Taste
   - Smell, fragrance
   - Emotion

- Movement

5. The Ultimate Goals of Hypnodrama: Building new Neuro-Pathways
   - We are re-training the brain to take a different direction in response to a stimulus
   - We are making 'unknowns' into 'knowns'
   - We reduce anticipatory anxiety
   - We break old associations and install new ones.

RESONATION

The reaction in a client when the therapist searches out and feeds back to the client, in hypnosis, specific words or symbols that 'resonate' with the client's own personal value system, experiences and goals. The client 'resonates' with the suggestion and accepts it.

Hypnodrama is commonly used for:

- Sports performance
- Examination and test anxiety
- Auditions
- Public speaking
- Fear of flying or other fears
- Preparing for court testimony
- Any behavior that the client wants to change

**Sports**

- Give a primary keyword.
- Give a secondary keyword to engage the 'second wind'.
- Coaches must participate in the process and know the keywords.
- Give a third key or anchor that allows a client to 'turn off' after the event.
- Emphasize that the subconscious mind knows exactly what to do – the minute the client 'thinks' about it, it's over – he loses.

**Oral Exams**

- Have the client use the phrase as an anchor, "Could you repeat the question please?"
- Have client ask, "Do you require more information?"

**Written Exams**

- General suggestion: "Because you are so relaxed, concentration intensifies, memory is stimulated, and recall is there. Now, thinking of relaxation, concentration, memory and recall, which do you think is most important?"
- "You will put your hands down on your desk, and you will read the directions."
- "If your mind goes blank, put your pen/pencil down, and when you pick it up again, the recall is there."

For all of the above situations

- Emphasize nutrition – avoid sugar drops during the event.
- Remember to reframe anxiety into energy or anticipation – make it work for the client. You may never take it away completely.
- "As you leave there's a smile on your face."
- Prepare client for the 'crash' afterward.

**Remember**

- Hypnodrama is easy!
- Hypnodrama is extremely effective.
- Go very slowly and give the client time to build the experience.
- Make the experience real. Make the client feel it.
- Let the client indulge in their new positive experience.

# CHAPTER 13
# HYPNOTIC REGRESSION

## AGE REGRESSION

Age regression is based on the model that everything that has ever happened to us in our lives is recorded in our minds like a tape recorder. Age regression therapy operates from the assumption that many of our problems as an adult stem from traumatic experiences that happened to us early in life and that were repressed from conscious memory by the subconscious defense mechanism of repression. Such a model is commonly held among traditionally trained psychologists and psychoanalysts schooled in the Freudian tradition.

Age regression is commonly associated with hypnosis. Many professionals use the terms hypnosis and age regression synonymously. Such professionals generally engage in what we refer to as “hypno-miracle”. Such a practice attempts to rapidly hypnotize the client, regress them back until “cause” is found, exorcise that “cause” and achieve a “cure” all in one session. However, the classical approach to age regression suffers from the following shortcomings:

## SYMPTOM SUBSTITUTION

Finding an isolated traumatic experience as the cause to a problem is the exception rather than the rule. More often than not the hypnotist **leads** the client into finding some early life trauma as the cause, which may or may not be the actual cause. The hypnotist then is using regression and the finding of the cause as more of a paradigm element than an actual curative process. If the hypnotist is successful at removing a symptom with this modality, one problem still remains. By not finding out what the “real” variables or “common denominators” were that created and/or supported the problem, those variables still remain and, therefore, the problem will either re-occur or be transferred into another symptom.

For example, let’s say that a person cannot sleep at night, and the hypnotist uses regression and leads the client into discovering a traumatic event when they were a child where they were awakened suddenly, and that fear stuck in their mind and made them afraid of sleeping deeply. Suppose, however, that the real factors were that the person is experiencing stress on the job, their diet is a factor, and they have a major financial decision to make that is worrying them. Even if the regression “modality” were successful in removing the symptom, what happens to the stress and effect of the other untreated variables? The on-going effects of those variables will either cause the problem to re-occur, or it will create another symptom. This has been an age-old criticism of hypnosis as a therapeutic tool.

## AVOID LEADING THE CLIENT

It is impossible not to lead the client. No matter how objective you think you are being, you will lead the client in some way. Most hypnotists enter into the therapeutic relationship with some underlying assumptions about the client, what caused the problem, or even merely the nature of human behavior. You wouldn't be doing an age regression in the first place unless you held the assumption that there is some trauma in the person's past that you believe is responsible for the person's symptom(s).

One of the biggest drawbacks to using regression as a fact-finding tool is that a lot, if not most, of what surfaces is not reality. The client, however, (and many times the hypnotist as well) confuses this material as fact and can develop many questions or doubts about their past, or about things that they think have happened to them. A common example of this is when the hypnotist or the client suspects child abuse or sexual abuse in the client's history. In regression, such suspicion most likely will lead the client into discovering such abuse, whether it really happened or not. Once discovered, it becomes difficult, if not impossible, to separate fact from fiction. Thus the client's belief that they were abused, when in fact they were not, creates a whole new set of problems for the client.

DEFENSE MECHANISMS

The purpose of age regression is to go back in the client's history and discover early life traumatic experiences.

Regression is required because, these experiences are assumed to be blocked from conscious memory by a subconscious defense mechanism known as repression. True repression, however, is a fairly significant defense mechanism and, as the name defense mechanism implies, it is there to protect the client from something that they are not ready to experience because it is too traumatic or painful. Repression is a strong defense mechanism, and should be treated with great respect when discovered.

Important pointers:

If a memory has been repressed from consciousness, it may not be the wisest or first choice to expose the memory, and the related experience, to the client without knowing that they are ready to handle it.

Another point to consider is how many traumatic events (repressed or not) does one have in one's lifetime.

One has to consider the wisdom of making someone very suggestible and then going back and re-living every traumatic event that ever happened to them.

SAFEGUARDS

When using the regression technique certain safeguards should always be in place. One is the "hidden observer". This technique allows the client to view past events as though they are just a hidden observer of the events, and not actually "there". This provides the client with some protection from the emotional impact of the event. The person can experience even further removal from the impact with the suggestions that they are watching it on a TV screen. The sound can then be turned down, or the picture made smaller. The hidden observer technique can also allow the client to remain as their "adult self" and even have the "adult self" provide narration or make comments, suggestions, or give supportive advice to the "child self" in the event.

The hypnotist must remain in total control of the situation, watching the client carefully to judge when the client is experiencing too much anxiety. Flooding the client with emotions can be anti-therapeutic, and should be avoided. By remaining in total control, the therapist can then instruct the client to "let go" whenever the emotions of a scene become too high. The client could then be redirected to be right back in the therapist's office in the recliner chair feeling safe, comfortable and relaxed.

The "psychotic swing" or other signs of personality instability should be constantly watched for. If the hypnotist has any doubts about the stability of his client, he should not perform any regression until receiving a second opinion from a health professional trained to understand personality disorders. It is also advisable that the hypnotist does a good history, and suggestibility conditioning process, before regressing a client. Doing a regression in the first session is not usually advisable. Get to know your client better first.

# CHAPTER 14
# FEAR & PHOBIA MANAGEMENT

## FEAR vs. PHOBIA

Fears and phobias are one of the most common reasons clients come in for therapy. It is very important to learn to distinguish between a fear and a phobia, because the treatment approaches are different.
**Fears:** An emotion of varying intensity aroused by a recognized and logical threat and involving the fight/flight response. It arouses a feeling of unpleasant tension and a strong impulse to escape, as well as certain physiological changes.

**Phobias:** An excessive, unrealistic and/or illogical and uncontrollable fear elicited by an object, situation, or activity. It usually involves strong physiological and emotional responses.

## FEARS

Most fears occur in childhood. Obviously, the child faces more unknowns and has less experience in reasoning powers. Added to this is a child's natural higher suggestibility. Most children grow out of their fears as they learn and adults explain things. However, some do carry fears over into adulthood and seek therapy. Others develop adult fears that often prompt them to come in for therapy.

In case of a fear, the primary cause of the fear stimulus is usually a known factor. The client can generally define the fear, and often knows when the fear started.

## PHOBIAS

It is more common for phobias to develop in adulthood.
In the case of a phobia, the client is often vague about the onset of the problem and is aware that the phobic reactions are irrational in nature. In extreme cases the phobic stimuli can trigger irrational and uncontrollable responses, which, in turn, exacerbate the problems. The individual, as a result, ceases to be able to lead a normal life.

Unlike a fear, a phobia generally develops, using the Law of Association, from an unknown primary cause. This difference is important, as will become evident when studying the different therapeutic approaches.

A general rule to follow, in the ini tial consultation, is: If it has a logical cause and started in childhood, this is one factor indicating a fear. If it originated later in life and the primary cause is unknown, look for other clues pointing toward a phobia.

Phobic stimuli (which are usually non-threatening themselves) are buried in the subconscious mind associated with the primary cause (which was threatening at the time) at a moment of extreme suggestibility.

## PHYSICAL REACTIONS TO PHOBIAS

With a phobia, the client does not know the cause but can explain what is happening. Some physical manifestations are:

- Muscular tension
- Cold hands and feet
- Respiration changes
- Hyperventilation
- Vocal cords tighten

## THINGS PHOBICS SAY

- I had a feeling something terrible was going to happen.
- I don't know why, but I was terrified.
- I thought I was having a heart attack.
- I thought I was going crazy.
- I thought I was going to die.

The phobic describes having "waves of panic."

## COMMON FEARS

- Fear of loss
- Fear of rejection
- Fear of success
- Fear of failure
- Fear of pain
- Fear of exposure
- Fear of strangers
- Fear of homosexuality
- Fear of sexual performance

- Fear of responsibility
- Fear of the unknown

## COMMON FEARS OR PHOBIAS

- Fear of death or dying
- Fear of flying
- Fear of heights (Acrophobia)
- Fear of public speaking (usually a fear, may be phobic)
- Fear of sexual dysfunction (more common with Emotionals)
- Fear of blood (Hematophobia)
- Fear of dirt or contamination (Mysophobia)
- Fear of animals
- Fear of impending danger (usually a phobia)
- Fear of water (Hydrophobia)
- Fear of closed spaces (Claustrophobia)
- Fear of open spaces (Agoraphobia)
- Fear of loss of control
- Fear of intimacy

## MAIN TOOLS FOR TREATMENT

When talking and exposing a problem, the venting process takes place, and the problem may diminish somewhat by itself.

There are three main tools for dealing with fears and phobias:

1. Symptomatic approach
2. Circle therapy (do not use if a phobia is suspected)
3. Desensitization

## SYMPTOMATIC APPROACH

This approach works well with physical suggestibles or somnambulistic clients. In this approach the therapist literally suggests the symptoms and problem away. It only works completely with a small percentage of clients.

The preferred tools for working with **fears** are Circle Therapy, and Desensitization.

The preferred tools for working with **phobias** are Desensitization, or Systematic Desensitization. Do **NOT** use Circle Therapy if a phobic condition is suspected.

## CIRCLE THERAPY

Circle therapy is a very strong and rapid venting process. It must NOT be used for treatment of a phobia. The rule is, if in doubt as to fear or phobia, don't use Circle Therapy. Circle Therapy can also be used for dealing with other traumas. The trauma in this case is created in a hypnotic state. **Make sure the client feels it!**

Let the trauma build (being very paternal if necessary), then tell the client to "Pass it, let it go, relax and go deeper". Then use a reversal with the suggestions of positive reinforcement. "You may try to feel it, but the harder you try, the more difficult it becomes. In fact, you begin to feel a new emotion, amusement, and a tendency to smile, even to grin." As the client begins to respond, strengthen the positive response and bring him out of hypnosis.

After the reversal, repeat the trauma. This time the trauma will not be as strong. When the client has had enough, reverse it again, reinforcing the positives. Continue the process until the client feels no fear in the situation. Typically, this takes about three times. About 50% of the time it can be handled in one session.

As the therapist, you will notice that each time the process is repeated, the fear and abreactions become less and less.

Generally, in order to get to the cause the therapist must do an age regression with the client. It is used to find out what it was that frightened the client at that time. The client has, in this case, pushed the primary cause inside, and it must be dealt with in regression.

If the fear cannot be dealt with in one session, use suggestions that the client will vent out the fear in dreams.

## DESENSITIZATION

At first glance, this appears to be similar to Circle Therapy. The important difference is that the client is **NOT** taken back to the trauma itself. He is only brought in touch with the fear response elicited by the fear stimulus and then, while in hypnosis, taught to **associate** the stimulus with a new and positive response, such as the calmness and relaxation of hypnosis. It is more of a conditioning process.

## SYSTEMATIC DESENSITIZATION

This is form of behavior therapy, developed by J. Wolpe and others, in which the client is trained, in a deeply relaxed or hypnotic state, to associate responses to phobic stimuli with new and positive responses. The various phobic stimuli experienced are ranked on a subjective distress scale. They are then presented and worked on progressively, starting with the weakest one first. The process effectively desensitizes the client to the anxiety provoking stimuli with early experience of success.

## FEARS OF FAILURE AND S UCCESS

A fear of success is more common than a fear of failure.

### Fear of Failure

People obsessed with a fear of failure tend to become very successful at some point in life. For example, a high emotional sexual, who is also obsessive compulsive, will tend to ride roughshod over all obstacles to accomplish what is required to achieve "success" while being totally insensitive to how they affect others.

### Fear of Success

Most of the reasons for a fear of success are a result of our early conditioning in life. An example of this is the feeling that the child has that they don't deserve success. Some of these reasons are:

- Lossofprivacy(toomuchexposure)
- Responsibilitiesinvolvedifsuccessful
- Associationofsuccesswithtrauma(successfulfatherwholo steverything)

These are some reasons. There are many others. The Mental Bank is a good therapeutic modality to use in case of "fear of success". The mind associates success with something that is not valid today. For example, income of people perceived as successful or not successful in childhood is subconsciously held up to today's inflated standards.

## ANXIETY

The two forms of anxiety are:

- Static
- FreeFlowing

**Static Anxiety**

This form of anxiety is related to the fear of success. It is always there and does not come and go. There is always a logical reason for the anxiety. Sometimes the client cannot explain a logical cause. This is because it is not always real. A lot of expectation is involved. Treat this like a phobia.

**Free Floating Anxiety**

This form of anxiety is related to the fear process and is intermittent. The client will be worried and fearful, which would get triggered off periodically. He is overly sensitive, which in turn leads to hyper-suggestibility.

**Treatment**

With anxieties, use the same approach as with fears and phobias. In other words, treat static anxiety in the same way as a phobia, and free-floating anxiety as a fear.

# CHAPTER 15
# PAIN MANAGEMENT

## OVERVIEW

**General Anesthetic vs. Hypnosis**

General anesthetic is more predictable and eventually will lead to the emergence of drugs for pain control.

Hypnosis offers a cure for functional medical disorders because of the release through the subconscious mind.

One problem today with hypnosis and medicine is that hypnosis is not understood by the medical profession.

Among younger doctors, however, Behavioral Medicine (Medical Psychology) is aiding in total medical cure through a team effort.

## REASONS FOR MEDICAL HYPNOSIS

**Indications (Positive)**

- Good suggestibility. Have medical backup available.
- Plan a contract. The client must agree to this.

**Contraindications (Negative)**

- Psychotic tendencies
- Drugs. We don't know the reaction under hypnosis. Certain drugs sometimes block receptors to the brain.
- So much pain that you (as a therapist) feel disturbed. A paranoid illness patient has the attitude – "Show me what you can do for me."

## INTRODUCING HYPNOSIS TO CLIENT

The first rule of dealing with medical hypnosis is to obtain an understanding of what you are doing. Find a common ground with the client, and use their belief system. Present hypnosis in such a way that you will never fail.

## PAIN

Pain is generally due to injury. Teach the client how to change the physical reactions to the body.

A side effect of painful stimuli is an increase in muscle tension. Muscle tension then leads to stress and anxiety. The body is primed to go into fight/flight from anxiety or stress.

After fight/flight, the body is primed to go back to stress and anxiety. One flight mechanism is depersonalization (thinking you are not yourself). The fight response triggers irritation and agitation.

Both fight and flight add more stress and anxiety. Also, a decrease in the flow of blood in the hurt area occurs, aiding in the release of toxins, which leads to an increase in the perception of pain (this creates an endless loop from pain to blood flow).

Under stress or pain, the client will regress to previous behavior patterns.

## AFTER "DEEP SLEEP"

This applies not just to the pain treatment but to everything the therapist does.

### Image Based Suggestions

- Imaginative Inattention – Imagining that you are not attending to anything special.
- Imaginative Transformation of Pain – Hypnoplasty – The remodeling of sensations under hypnosis. This would include remodeling the pain sensation into regular heat or cold.
- Imaginative Transformation of Context – Context in which the injury or pain occurred. Make the client feel like a hero (even if they are a martyr). Create another mental set, or reason for the pain (such as work related injury becomes the injury of a sports hero).

### Non-Image Based Suggestions

- Focus Attention on Physical Characteristics of the Environment – To help the client take their mind off the pain, have them count dots in the ceiling, lines in the wall, or anything else that comes to mind.
- Mental Distractions – Have the client take their mind off the pain by doing mental arithmetic, listening to music, etc.
- Suggestions – Focus Attention in a Disassociative Manner on how deep the pain goes, is it left or right side or both? Find out where it does NOT hurt.

### Other

- Link Image and Non-image suggestions together.
- Passive Concentration – This includes ideas such as "Concentrate on the sound of my voice, and the sound of my voice only."

## HEADACHES

There are at least 75 different types of headaches. Some are physiological in origin.

**Migraine Headache**
This is a vascular headache and occurs only one or two times per month. The headache will generally last 24 to 36 hours. Most migraine headaches have a warning system of some sort. They are not very common.

**Muscular Contraction Headache**
This is the more common variety. There are basically three types:

1. Acute – Occurs three to six times per year. This is the standard type headache that most people take an aspirin for, and it goes away.
2. Chronic – This type of headache appears to last forever! The client will not respond to drugs etc.
3. Conversion Headache – This is not a physical pain. The therapist should give the client a "bandage" unless the cause of the headache is found.

**BACK PAIN**
A sedentary lifestyle, an over-active weekend, or a beer belly could cause a back pain. In conversion back pain; a lot of "stuff" gets dumped down there. Breathing exercises may help break the pain cycle.

**INTESTINAL PAIN**

Even if the client is emotionally sound, if the pain is bad enough, changing electrolytes will cause emotional changes (chemical imbalance).

# CHAPTER 16
# NEURO LINGUISTIC PROGRAMMING (NLP)

## OVERVIEW

Neuro-Linguistic Programming is a model about human behavior. It is not a theory because a theory must be proved. On the other hand a model merely has to be tested and if the model yields consistent results; it qualifies as a working model.

Every model is based on pre-suppositions which are assumed to be true. The pre-suppositions for any given model are fine-tuned till such time that the model yields consistent results.

1. Everyone lives in and operates from his/her own unique model of the world.
2. People always make the best choices available to them, given their unique model of the world and the situation.
3. There is a desirable solution/possible outcome to every problem.
4. Each person is equipped with everything he/she needs to solve his/her problems.
5. It is important to separate and distinguish a person from his/her behavior. When someone is learning something new, it is useful to evaluate the behaviors while holding constant a positive evaluation of self.
6. All behaviors that people exhibit are motivated by a positive intention or purpose.
7. All communication has a desired intent and ultimate response. The meaning of a communication results from the response it elicits, not from its desired outcome intended. Intention behind a communication is not its meaning.
8. Control of the outcome of interpersonal interactions goes to the person with the greatest variety of behaviors and the most flexibility in using them.
9. The same neurological circuits are used for memory and imagination. Therefore, they have the same potential impact.
10. Everyone's unique model of the world, including their knowledge, thought, memory, and imagination is the result of how they combine and sequence information in their representational systems.
11. The mind and body operate as a system. Any change in one affects the other.
12. If what you're doing isn't working, do something different.

## INTRODUCTION TO NLP

NLP is a model of the illusion of human behavior and communication – the study of the components of perception and behavior, which make our experience possible. The name Neuro Linguistic Programming stands for the basic process used by all human beings to encode, transfer, guide and modify behavior.

**"Neuro"** stands for the fundamental tenet that all behavior is the result of neurological processes.

**"Linguistic"** indicates that neural processes are represented, ordered, and sequenced into models and strategies through language and communication systems.

**"Programming"** refers to the process of organizing the components of a system to achieve a specific outcome.

## THE NLP PROCESS

We all indulge in a certain behavioural pattern with a basic desire to obtain a favourable outcome for ourselves. The probability of a favourable outcome increases sharply when the person in our environment with whom we are interacting accepts us as a part of his natural set-up. This in turn no longer makes us a potential threat to him. Once we gain such acceptability, it now becomes possible to steer the interaction to a desired favourable end point.

NLP was founded by behavioral modelers John Grinder and Richard Bandler to analyze and explore the patterns governing the complex process of human behavior. The basic premise of NLP is that there is a redundancy between the observable patterns of human behavior (for example, linguistic phenomena, eye movement, hand and body position, and other types of performance distinctions) and patterns of the underlying neural activity governing the body.

NLP is a process and, at the same time, a model of a process. All of the information, laws, patterns and conclusions made by the model about past experience are only important or useful in the way that they relate to one's immediate ongoing experience. This is especially important to keep in mind when one is dealing with another system that is subject to many levels of change, depending on its interaction with itself, its social environment, and its ecological environment.

## A Basic Outline of the NLP Process

Train and expand one's own sensory awareness of other human beings in order to:

1. Observe and identify meaningful patterns of behavior that are systematic, recurring, and part of everyone's sensory experience.
2. Notice what responses one's own choice of behavior elicits in oneself and other human beings.

**Utilize the information one gathers through these observations to determine:**

1. The representational distinctions human beings can make about their internal and external experience, i.e. their ability to see/visualize, hear/verbalize, etc.
2. Patterns involving the combination and connection of neural networks (sensory representation) underlying behavioral processes.
3. How these distinctions and patterns affect the strategies people use to organize, and communicate about their sensory experience and internal maps.
4. How these distinctions, patterns, and strategies may be utilized to understand and promote the processes of learning, communication, motivation, and choice in human beings within the individual and his/her social and ecological environments.

The mechanics of this process may be generalized into the following basic procedure:

1. An outcome is identified that is mutually acceptable to the facilitator and the client. Explicit criteria for the achievement of the outcome are delineated.
2. One individual generates a communication, in the form of verbal and non-verbal behavior, in an attempt to direct or propel the client to the desired outcome. This communication elicits a response in the form of some access of information and return information on the part of the other individual. The interaction will take place in both the verbal and non-verbal level of response.
3. The two individuals may then work together to make distinctions in the client's experience and response, both internally and externally.

a. Via the facilitator's perception of the client's external behavior within the specific contextual setting.
b. Via discussion and questioning about the client's internal experience.

4. The distinction may then be classified and segmented into a formula that traces the client's behavior in terms of relevant input, mental processing, and behavioral output.
5. The information provided by these patterns and distinctions is utilized to make decisions and prediction about the individual's current and future behavior, and to devise more effective strategies that the client can use to achieve desired outcomes.
6. The facilitator then varies his behavior in accordance with this information, and the process repeats itself until the desired outcome is reached.

## PACING AND LEADING

Pacing is the term given to the process used to establish and maintain rapport on a physiological level. It utilizes techniques of mirroring and matching another person's behaviors in such a way that they are not conscious of the process, and the unconscious message they receive is that you are somehow similar to them. Pacing is a way to establish rapport very quickly and maintain it effectively, so that your communication with others is successful. There are several methods of pacing:

1. Whole Body Matching: Adjust your body to match the other person's stance or position.
2. Half Body Matching: Match the upper or lower portion of the other person.
3. Partial Body Matching: Match any consistent or stylistic use of the body, such as hand movements, shrugs, eye blinks, etc.
4. Facial Expressions: At times match the way the person uses his/her face.
5. Gesture Matching: Match gestures of the other person with smaller ones.
6. Voice: Match tone, tempo, intensity, volume, rate of speech.
7. Representational Systems: Pace in your own language the predicates used by the other person.
8. Breathing: Adjust your breathing to match the other person's breathing.

9. Crossover matching: Use any element of your behavior to match another element of the person's behavior. For instance, you can move a finger to the rate of their eye blinks, or adjust your rate of speech to their breathing.

**Major Presuppositions of NLP**

There will be exceptions to all these presuppositions, but they are useful starting basis for communication and personal evolution.

1. **Communication is redundant:** You are always communicating in all 3 major representational systems.

2. **The meaning of your communication is the response that you get:** Communication is not about what you intend or about saying the right words; it's about creating an experience in and getting a response from the listener. The bottom line is the response you client.

3. **The map is not the territory:** People respond to their map of reality; not to reality itself. NLP is the science of changing these maps (not reality).

4. **Requisite variety:** The element in a system with the most flexibility will be the controlling element.

5. **People work perfectly:** No one is wrong or broken; it's simply a matter of finding out how they function now, so that you can effectively change that to something more useful or desirable. People don't need to be fixed.

6. People always make the best choice available to them at the time : ( but often there are lots of other better ones)

7. Every behavior is useful in some context.

8. Choice is better than no choice.

9. **Just about anyone can learn to do anything:** If one person can do something, it is possible to model it and teach it to anyone else.

10. **People already have all the resources they need:** What they need is access to these resources at appropriate time and places.

11. There is no such thing as failure, only feedback: Every response can be utilized.

12. **Chunking:** Anything can be accomplished (by anyone) if you break the task down into small enough chunks.

13. **Behind every behavior is a positive intention:** While every behavior may be harmful or seem "bad", there is always a positive intention behind the behavior.

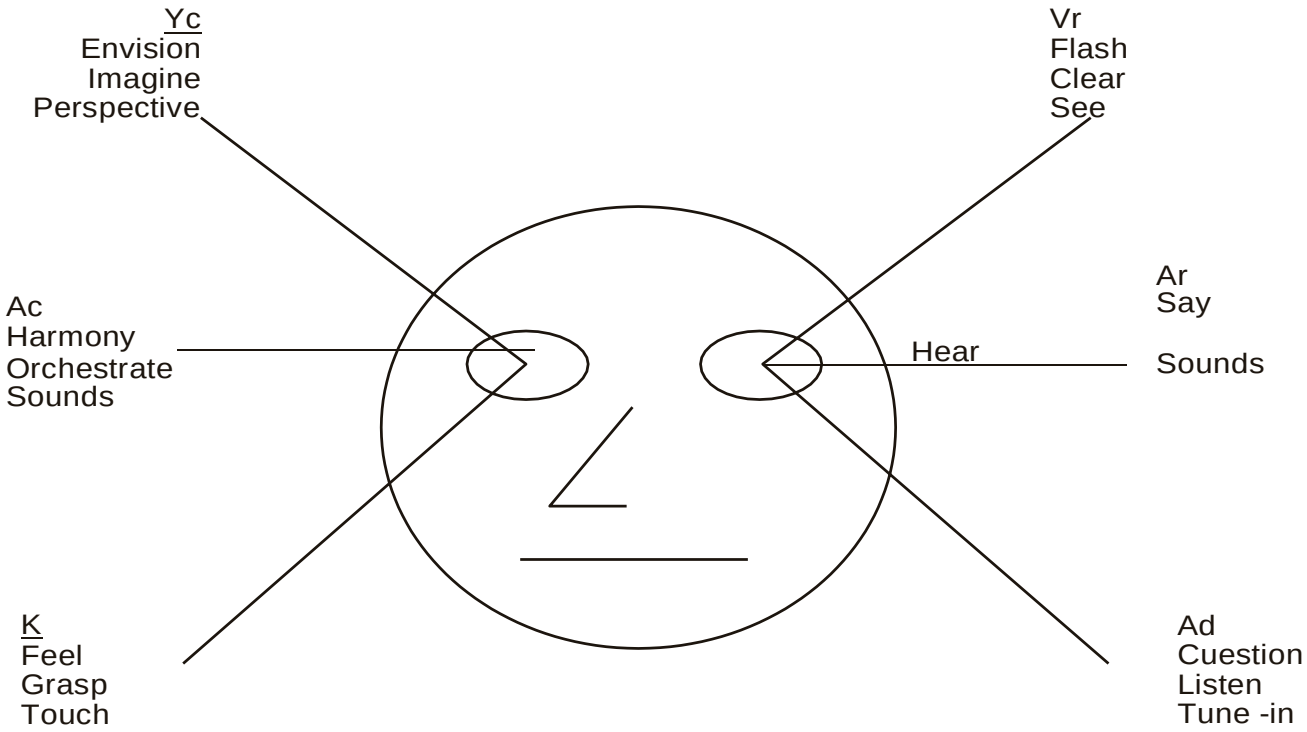
14. **Symptoms** – pain, anxiety, depression, tumors, colds, etc are communications about needed action.

15. **We are all responsible for creating our own experience:** Even when challenging events that we cannot control happen, we are responsible for our responses to these events. Typically, however, we have much more control than we think we have. Another way of stating this presupposition is that "We consistently create our own environment" through our beliefs, filters, capabilities and behaviors.

## REPRESENTATIONAL SYSTEM PREDICATES

| Visual | Auditory | Kinesthetic | Gustatory/ Olfactory |
|---|---|---|---|
| hazy | hear | feel | taste |
| obscure | listen | warm | flavor |
| dim | talk | touch | savour |
| film | harmony | handle | relish |
| opacity | noisy | grasp | tangy |
| perspective | call | soft | palatable |
| cloudiness | loud | tight | aftertaste |
| focus | shout | smooth | odor |
| picture | told | rough | smell |
| envision | dissonance | firm | scent |
| vision | resounding | pressure | whiff |
| viewpoint | lend an ear | tense | pungent |
| view | amplify | concrete | stink |
| look | cadence | hurt | reek |
| gaze | chatter | roughly | a nose for |
| illuminate | whine | clumsy | leave a bad taste |
| glimpse | moan | relaxed | |
| illustrate | hiss | swell | |
| perceive | groan | tremble | |
| watch | voice | shiver | |
| scan | acoustics | shake | |
| survey | sounds like | penetrate | |
| visible | cry | absorb | |
| glance | silence | grope | |
| glare | tune | stir | |
| stare | tone | agitated | |
| show | scream | cutting | |
| pretty | | glow | |
| picture | | flush | |
| clear | | itch | |
| glow | | creeps | |

# EYE ACCESS MOVEMENTS FOR A "NORMALLY ORGANIZED" RIGHT-HANDED PERSON (FACING YOU)




From Subject's point of view:

Vc: Visual Construct Images (eyes up right)

Vr: Visual Recall (Eidetic) Images (eyes up left)

AC: Auditory Construct Sounds or Words (eyes horizontal) and to the right)

Ar: Auditory Recall Sounds or Words (eyes horizontal and) and to the left)

K: Kinesthetic Feelings/Smell, Taste (eyes down right)

Ad: Auditory Recall Sounds or Words (eyes down left)

# EYE MOVEMENTS

Visual
Contruct

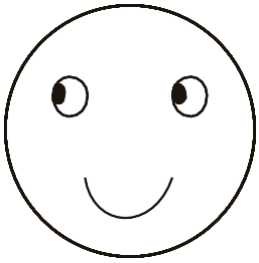
**new Pictures**

Visual
Remembered

**old Pictures**

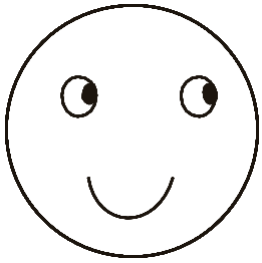
Auditory
Construct

**compose**



Auditory
Remembered

**recite**



Kinesthetics
(tactile & emotions)

**feelings**

Internal
Dialogue

**talk to self**

***Organized Right-Handed***

## CODE OF CONDUCT FOR ALL STUDENTS / THERAPISTS

1. We are primarily learning Integrated Clinical Hypnotherapy for Self-focus and Self-Empowerment.

2. We are the Lords and Masters of our Universe. Our Universe is a manifestation / projection, consciously or unconsciously of ourselves.

3. Therapy starts with ourselves and we do not blame others or situations for what we manifest in our Universe.

4. We will only attract the clients that we are ready for and are eligible to handle.

5. We inform clients adequately about the therapy and its limits.

6. We do not misrepresent our professional qualifications, affiliations, and purposes. We disclose, upon request, our training, qualifications, and competencies to prospective clients.

7. We do not reject clients on the basis of gender, race, ethnic background, culture or sexual preference.

8. Our reception and our therapy room are appropriate. Our personal hygiene and the hygiene of our therapy room are impeccable.

9. We are responsible for the therapy, but never act against the best interest of our clients, or against their express will. However, we cannot shirk responsibility by claiming to follow a request from a client. The welfare of the client is the prime consideration at all times. When the client's desires and our professional responsibility conflict, we will end the therapeutic relationship courteously. We do neither blame the client nor ourselves for that.

10. As we deal with intimate thoughts, feelings and details of our client's life, we retain a professional distance. We present our work and ourselves professionally. We do not behave as personal friends or acquaintances. We do not engage in intimate relationships of any kind with clients.

11. We recognize transference, counter – transference and other projections, and seek to restrain these. We don't assume the role of Helper or Savior; we discourage admiration and deference from the client. We avoid making clients dependent on us; we stimulate their own responsibility and independence.

12. We do not try to convert clients to any belief or practice. We respect their convictions, though we may point out how these seem to relate to their problems, if this seems the case.

13. We build a relationship of trust with the client and will not betray that trust. We will treat all information from clients confidentially and will not divulge it without the full consent of the client, except when obliged by law to do so. In publications we protect the anonymity of the client or ask written permission.

14. We only perform those therapies for which we are qualified. Unless otherwise licensed to do so, we do not provide or offer to provide diagnosis or treatment which is limited by law to appropriately licensed practitioners.

15. We stay alert if clients need anything outside our own experience or training. We then refer to colleagues or suggest other suitable methods outside our field.

16. We advise clients to see a doctor when problems could have medical cause. We never suggest decreasing or stopping medication. We limit ourselves to psychosomatic diseases and physical complaints for which no physical cause has been found. We do not pretend to heal serious and terminal diseases, but at best to influence favorably the course of such diseases.

17. Sessions are only recorded with the consent of the client. The client may withdraw his consent rights after the session. Clients are entitled to copies of records of sessions.

18. We refrain from making disparaging remarks to clients about colleagues or other therapies in the regular or alternative field.

19. We respect and safeguard the professional works and original ideas of others, and give full credit and appropriate citation for such works and ideas when used in our publications and presentations.

20. We are willing to discuss our professional activities with fellow members. We collaborate with complaint procedures.

21. We will assess our own competence, skills and limitations. We seek additional knowledge and skills through participation in seminars. Workshops or trainings and other opportunities. We keep informed about developments in the profession and in society.

22. We will collaborate with any serious research that does not prejudice the therapy process. We adhere to the generally accepted academic guidelines in conducting research with people.

23. We exercise prudence and discretion in promoting, encouraging, and facilitating public awareness and understanding of the field. We avoid sensationalism, inaccuracy or misleading claims.

24. We adhere to professionally accepted guidelines and standards of good taste in informing the general public of the availability of our services.

25. We seek to protect the public from all those who are not qualified to practice by deficient education, preparation, competence, emotional stability, or ethical standards.

26. When we observe inappropriate professional practices of fellow members, we will, whenever possible, discuss this with them. If this does not work, we will inform CHII in writing.